

REGIONAL MEDICAL CENTER

DEPARTM

PATIENT

Name :

ADMITTING DIAGNOSES (ICD-10-CM)

M16.1

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HISTORY

CHIEF COMPLAINT

Severe

HISTORY OF PRESENT ILLNESS

Mrs. Ha

Over the past 18 months she has developed progressive right groin

pain wi

She has failed conservative management:

- 4 m

IMAGING (Regional Medical Center Imaging Study RMC-IMG-882441,

2024-09

PAST MEDICAL HISTORY

- COP

PREOPERATIVE OPTIMIZATION (Pulmonary Medicine, R. Nazari MD,

consult

CARDIAC RISK ASSESSMENT

RCRI sc

MEDICAL NECESSITY DETERMINATION

Per CMS

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OPERATI

SURGEON: M. Petrov, MD

ASSISTA

INDICATIONS

Symptom

PROCEDURE DETAILS

After c

A posterior (Moore / Southern) approach was used through the prior

surgica

The bipolar femoral head was removed using the manufacturer-provided

extract

Trial reduction with a 54 mm cup, 36 mm head, +5 mm neck length

demonst

Final components implanted:

- Ace

The wound was irrigated with 3 liters of pulse-lavage saline

contain

INTRAOPERATIVE CULTURES

Aerobic

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PROGRES

POD #1 (2024-09-23, M. Petrov MD)

- Pat

POD #2 (2024-09-24, M. Petrov MD)

- Pat

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OPERATI

SURGEON: M. Petrov, MD

ASSISTA

INDICATIONS

Symptom

PROCEDURE DETAILS

After t

A modified anterolateral (Hardinge) approach was used. Sharp and

blunt d

A femoral neck osteotomy was performed at the templated level. The

femoral

Acetabular reaming was performed sequentially from 50 to 54 mm. The

femur w

Final components implanted:

- Ace

Trial reduction and final reduction demonstrated excellent stability

with re

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DISCHAR

HOSPITAL COURSE

Bilater

PROCEDURES PERFORMED THIS ADMISSION (as billed)

- 202

NCCI / BILLING ATTESTATION

The two

DISCHARGE DIAGNOSES (final)

- M16

DISCHARGE MEDICATIONS

- Asp

DISPOSITION

Dischar

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ATTENDI

I personally performed both procedures, supervised the entire

operati

Electronically signed:

[End of medical record.]